

clinIQ GTM Playbook V2.3

US, UAE, UK and Saudi Arabia clinic-operations growth motion

V2.3 Research Edition

Evidence-led strategy. No revenue forecast. Internal validation required where noted.

Executive understanding

clinIQ is best understood as an operational layer around the existing EHR/PAS. It combines real-time patient flow, digital check-in, scheduling, analytics, RTM billing, pre-authorization, telehealth, messaging, patient app/portal, wearable integration and secure file exchange. India is excluded from this GTM because clinIQ's current public locations are United States, UAE, United Kingdom and Saudi Arabia.

1. Commercial thesis

- Sell an operational problem first, not a feature tour: capacity, front-desk friction, prior-auth workload, RTM workflow, no-show leakage or multi-location standardization.
- Use SPICED early to diagnose business impact. Use MEDDPICC for larger multi-site/enterprise deals. Apply Challenger principles when the incumbent EHR creates 'we already have a system' inertia.
- No demo before diagnosis. A demo should prove only the 2-3 workflows that matter to the customer's stated business problem.

2. ICP and buying committee

Segment	Best-fit profile	Economic buyer	Technical / compliance buyer	Champion
SMB	Specialty practices with 1-10 providers and visible front-office or RTM/pre-auth friction	Practice owner / physician	Practice IT / EHR admin	Practice manager
Mid-market	10-100+ providers, multi-location specialty groups, behavioral health, PT/pain, urgent care	COO / Managing Director / VP Operations	CIO / Health Informatics / IT	Regional / Clinic Operations
Enterprise	Ambulatory networks or health systems where a contained specialty/site pilot is possible	COO / VP Ambulatory / Patient Access	CIO / Integration / Security	Operational service-line leader

3. Trigger-based account intelligence

Trigger	What to verify	Commercial hypothesis	First question
New clinics / acquisition / multi-site expansion	Official expansion announcement, provider hiring, acquired locations	Standardizing patient journey across sites may become harder	How are you standardizing check-in, flow and capacity visibility as locations expand?
Behavioral health / PT / specialty growth	New locations, service-line growth, telehealth + physical integration	Scheduling, no-shows and multi-site workflow complexity may increase	Where does capacity or patient access break as the service line scales?
Patient access / RCM / EHR initiative	Leadership hire, pre-auth initiative, EHR project, access program	Existing system may not solve the operational layer around the encounter	Which workflow remains manual around your EHR despite the current initiative?

4. Competitive intelligence

Competitor / status quo	Where stronger	clinIQ wedge	Where not to compete
Phreesia	Scale, enterprise proof, broad patient access/payments footprint	Whole clinic-operations layer including live flow plus RTM/pre-auth where validated	Do not claim broader overall platform without account-specific proof
Clearwave	AI patient engagement, scheduling, eligibility, check-in, payments	Operational floor visibility + workflow combination + specialty deployment	Do not frame Clearwave as 'just a kiosk'
EHR / PM native workflow	Bundling and incumbent trust	Solve operational gaps without replacing the record system	Do not assume every EHR lacks equivalent capability
Manual / spreadsheet / whiteboard	Zero new vendor cost	Measured standardization and visibility	Do not lead with technology; quantify the operational cost first

5. Economic impact / business case scoping

Leak / metric	Customer input	Calculation approach
Unused appointment capacity	Available slots, utilization gap, operating days, contribution/visit	Unused slots x operating days x net contribution per visit
Administrative burden	Hours/week on intake, calls, prior-auth, tracking; loaded hourly cost	Weekly hours x loaded rate x 52
No-show / cancellation leakage	Missed visits, realistically recoverable %, contribution/visit	Missed visits x recoverable % x contribution
RTM opportunity	Verified eligible patients, payer-specific reimbursement, clinical delivery cost	Eligible patients x verified net reimbursement - delivery/technology cost

Evidence rule

Vendor-published figures such as RTM reimbursement ranges or wait-time improvement are context, not guaranteed economics. Replace them with customer-specific numbers during discovery.

6. Land -> Expand -> Transform -> Retain

Stage	Commercial move	Trigger to next stage
LAND	One site, specialty or workflow: patient flow, check-in, RTM or pre-auth	Baseline success and user adoption
EXPAND	Additional modules, providers and sites	Multi-location demand, workflow standardization need
TRANSFORM	EHR/API integration, healthcare data/AI engineering, cloud modernization	Customer-specific requirements outside standard product scope
RETAIN	Customer success, workflow optimization, managed infrastructure where relevant	Ongoing operational ownership and expansion roadmap

7. Value-first outreach

- Fact -> Hypothesis -> Discovery Question is mandatory. Never turn a public expansion trigger into an invented wait-time problem.
- Micro-value assets: Clinic Operations Assessment, patient-flow bottleneck map, pre-auth process map, RTM eligibility/business-case worksheet.
- Executive hook example: 'You are expanding outpatient capacity. Before we discuss software, I would like to understand where the patient journey becomes harder to standardize as locations scale.'

8. Market opportunity indicators

Market	2025 category indicator	Commercial reading
United States	Patient engagement is a large and fast-growing digital-health category; North America held 43.3% of the global category in 2025	Primary market. Use category growth as demand context only, not clinIQ revenue.
UAE	Patient engagement solutions: \$168.6M in 2025; 23.6% CAGR forecast 2026-2033	Priority market for private groups and digitally ambitious clinics.
United Kingdom	Patient engagement solutions: \$907.8M in 2025; 20.9% CAGR forecast 2026-2033	Secondary market requiring NHS/private segmentation and procurement validation.
Saudi Arabia	Active public clinIQ location; market sizing to be validated in the account plan	Treat as product-active but validate current sales priority with CEO.

9. Commercial readiness checklist

Area	Status to confirm internally
Referenceable customers / quantified outcomes	CEO Input Required
Security / HIPAA documentation	Validate exact current artefacts
EHR integration depth by vendor	Validate production readiness and dependencies
Enterprise pricing / discount guardrails	CEO Input Required
Implementation capacity and support model	CEO Input Required
Renewal, adoption and expansion metrics	CEO Input Required

10. Target-account profile template

- Company / geography / specialty / location count / provider scale
- Verified trigger with source and date
- EHR/PAS / relevant technology clues
- Economic buyer, technical buyer, operational champion
- Fact, commercial hypothesis, discovery question
- Primary entry offer and proof required
- Competitor / incumbent / build-in-house / do-nothing
- Land, expand, transform, retain hypothesis

11. Real target-account intelligence examples

These are research targets based on current public signals. They are not confirmed opportunities or evidence of buying intent.

Aster DM Healthcare, UAE

Fact: In February 2026 Aster announced AED 1 billion of UAE expansion, including two new Dubai hospitals, additional capacity, and further digital/HIS and AI investment.

Commercial hypothesis: A rapidly expanding outpatient footprint may create a need to standardize patient-flow, front-office and operational visibility across locations.

Discovery question: As new facilities and capacity come online, how are patient-flow and front-office workflows being standardized across the network?

Universal Health Services / Talkspace, US

Fact: UHS completed its acquisition of Talkspace in August 2026, combining virtual behavioral-health capabilities with UHS facility and outpatient infrastructure.

Commercial hypothesis: A broader virtual-plus-physical behavioral-health continuum may create patient-journey, scheduling and operating-model integration questions.

Discovery question: As virtual and physical behavioral services come together, where is the patient journey hardest to keep consistent across settings?

Enterprise Monetization & Account Expansion Architecture

Industry Focus

Primary industry/care-setting focus: multi-location outpatient and specialty clinic networks, behavioral health, addiction medicine, psychiatry, pain management, physical therapy/rehabilitation, surgery/procedure centers, urgent care, primary/specialty care, FQHC/community and rural health. Current public locations include US, UAE, UK and Saudi Arabia. India remains excluded.

Documented pricing architecture

Pricing / packaging element	What it means commercially
Starter: \$249/month, up to 500 patients/month	Documented self-service/package anchor; not the enterprise story.
Professional: \$499/month, up to 2,000 patients/month	Adds RTM, scheduling, analytics and priority support; useful for proving operational value.
Enterprise: custom, unlimited patients	Adds pre-authorization, custom integrations, dedicated success manager and SLA. This is the enterprise expansion path.
Implementation: \$750 one-time	Low implementation barrier; enterprise implementation effort still requires internal validation.
EHR integrations: custom quote	Important enterprise value driver because integration complexity can expand scope.

Enterprise entry and proof

Step	Commercial design
1. Trigger-led assessment	Clinic Operations & Revenue Assessment around patient flow, intake, RTM, pre-auth or multi-site standardization.
2. Contained enterprise entry	One location, specialty, module or operational workflow with a measurable baseline.
3. First proof	Customer-specific evidence: workflow time, capacity

	utilization, administrative burden, RTM/pre-auth process performance or adoption.
4. Enterprise graduation	Additional sites/providers/patient volume, pre-auth, custom EHR/API work, SLA/support or broader operational standardization.
5. Account depth	Healthcare integration, data/AI engineering, cloud or managed infrastructure only when customer requirements genuinely extend beyond the product.

Enterprise value drivers and expansion triggers

Value driver / trigger	Enterprise implication
Patient volume	Public tiers already scale by patients/month; large volumes push toward Enterprise.
Locations and providers	Multi-site standardization and roll-out create expansion without relying on per-seat pricing.
Modules / operational scope	Patient flow -> RTM -> pre-auth -> analytics / other modules where relevant.
EHR/API complexity	Custom integration is separately quoted and can create healthcare-engineering adjacency.
Enterprise support / SLA	Dedicated success and SLA are explicit Enterprise requirements.
Expansion trigger	New sites, acquisitions, new specialties, EHR change, RCM/RTM initiative, standardization need.
CEO Input Required	Current Enterprise ACV, implementation margin, discounts, renewal/expansion history, support cost.

Enterprise sales rule

Do not lead with the lowest public price. Lead with the business problem, prove value in a contained scope, then expand only when customer scale, complexity, governance, integration or operational ownership creates a legitimate enterprise requirement.

Do not invent ACV, gross margin, discount bands or large-contract claims. Where these are not documented, mark them Executive Input Required.

Updated pricing source: clinIQ official Pricing page, accessed Aug 2026 - \$249 Starter, \$499 Professional, Enterprise custom, \$750 implementation, EHR integrations custom.

Source notes

[1] clinIQ official Features page, accessed Aug 2026.

[2] clinIQ official Pricing page, accessed Aug 2026.

[3] Grand View Research, Patient Engagement Solutions Market 2026-2033.

[4] Grand View Research, UAE Patient Engagement Solutions Market, updated Jun 2026.

[5] Grand View Research, UK Patient Engagement Solutions Market, updated Jun 2026.

All market figures are category-level indicators, not The Algorithm revenue forecasts. Vendor outcome claims require customer validation.